

DISCHARGE SUMMARY

PATIENT INFORMATION

Name: Patient 01
Age: 68 years old | Sex: Male
MRN: 000-TEST-01
Admission Date: [admission date] | Discharge Date: [discharge date]
Facility: Metro Medical Center, Cardiology Unit

PRIMARY DIAGNOSIS

Congestive Heart Failure with reduced ejection fraction (HFrEF), NYHA Class II-III

SECONDARY DIAGNOSES

- Hypertension, controlled
- Chronic Kidney Disease Stage 3a (eGFR 45-59 mL/min/1.73m²)
- History of coronary artery disease s/p stent [prior year]

HOSPITAL COURSE

Patient 01 presented with progressive dyspnea on exertion (DOE) and orthopnea x 3 days. Initial vital signs: BP 156/92 mmHg, HR 94, SpO2 92% on RA. Echocardiography revealed EF of 28% (down from 35% on prior study 6 months ago). Labs notable for BNP 487 pg/mL (elevated), Cr 1.8 mg/dL, K+ 4.3 mEq/L. Patient was euvolemic on exam without significant peripheral edema or JVD at discharge.

Treatment course included IV furosemide diuresis over 48 hours (total 240 mg) with resulting weight loss of 8 lbs and resolution of orthopnea. Patient remained hemodynamically stable throughout. Daily weights monitored; no acute decompensation. Discharged on optimized oral regimen after tolerating PO intake without nausea/vomiting.

DISCHARGE MEDICATIONS

| Medication | Dose | Route | Frequency | Duration |

|---|---|---|---|

| Furosemide | 40 mg | PO | BID (morn & eve) | ongoing |

| Enalapril (ACEI) | 10 mg | PO | BID | ongoing |

| Metoprolol tartrate | 50 mg | PO | BID | ongoing |

| Spironolactone | 25 mg | PO | q.d. | ongoing |

| Atorvastatin | 80 mg | PO | q.d. (evening) | ongoing |

| Aspirin | 81 mg | PO | q.d. | ongoing |

| Potassium chloride | 20 mEq | PO | q.d. | ongoing |

ACTIVITY & RESTRICTIONS

- No strenuous exertion or heavy lifting (>10 lbs)
- Ambulation as tolerated indoors; may walk outdoors on level ground
- Avoid prolonged standing or Valsalva maneuver
- Sexual activity permissible if no dyspnea with exertion

DIETARY RECOMMENDATIONS

Sodium-restricted diet: <2 grams/day (no added salt, avoid processed/canned foods, deli meats). Fluid restriction: 1.5 L/day (including beverages). Daily weight log required—notify MD if gain ≥ 3 lbs in 24 hrs or ≥ 5 lbs in 1 week.

FOLLOW-UP APPOINTMENTS

1. Cardiology clinic follow-up: in 2 weeks (repeat echo anticipated in 4-6 weeks)
2. Primary Care: in 1 week (BP/medication tolerance check)
3. Nephrology: in 4 weeks (CKD monitoring, renal function trending)

RED FLAG SYMPTOMS — SEEK EMERGENCY CARE IF:

- Acute onset dyspnea, chest pain, or syncope
- SpO₂ <88% or persistent tachycardia >120 bpm
- Sudden weight gain >